A Care Plan Needs More Than a Place

A short guide to coverage, backup and who follows up.

You know where your husband will be after the move. What nobody has finished saying is who will be with him, on which days, at which hours, and who steps in when that person can't come.

This guide explains the ideas behind those questions, so you can describe the plan the way the care team needs to hear it. It explains the ideas. It doesn't judge the plan.

Look at the plan as hours, not as a place

A place answers where. It doesn't answer who, or when. The first job is to set what your husband will need help with beside the help that has actually been arranged.

Medicare's discharge checklist gives families a starting list. It asks them to mark the everyday activities that need help and to discuss their concerns with the staff: bathing, dressing, using the bathroom and climbing stairs; cooking, food shopping, house cleaning and paying bills; driving to appointments and picking up prescriptions. [1]

Much of that help comes from family and friends. The federal Administration for Community Living says about 80 percent of care at home is provided by unpaid caregivers, which is a good reason to be exact about which relative, and when. [2]

- Put each kind of help beside a part of the day: early morning, morning, midday, afternoon, evening, overnight.
- Look twice at the night and at the handoffs, the minutes between one person leaving and the next arriving.
- A filled-in page describes the plan. It isn't a safety clearance, and nobody in the family can give one.

Describe where he'll be, and when he'd be on his own

You aren't inspecting anything. You're writing down specific things you've seen or been told. If he is going home, the CDC's home checklist for older adults shows the kind of detail that is easy to note: objects on the stairs, a handrail on only one side, throw rugs, cords across a walkway, a dark path from the bed to the bathroom. [3]

The judgment belongs to someone else. One of the questions the Family Caregiver Alliance suggests asking the discharge team is whether someone will come to the home to do an assessment and see if modifications are needed. [4]

- Write the concern, where it is and when it comes up: “the hall is dark at night,” not “the house isn't safe.”
- Mark the hours he would be on his own. Whether those hours are a concern is a question for the people who assess him.
- List the help that exists today, by name: who comes, what they do, which days.

Coverage, backup and what one person can carry

An offer and coverage are different things. “I'll help” is kindness. “Tuesdays and Thursdays, nine to two” is coverage. That distinction is the Family Care Roadmap's own, and this step turns on it.

The Family Caregiver Alliance's advice on asking for help points the same way: when you break the jobs down into very simple tasks, it is easier for people to help, and it suggests preparing a list of things that need doing. [5]

Backup is the answer to “and if that person can't come that day?” AARP's planning guide says team members need not all live nearby or have huge blocks of time to be of value, and that a deep, wide team helps make sure you still have support as people's ability to help changes. [6]

Your own limits go on the page too. The Family Caregiver Alliance says discharge planners should discuss with you your willingness and ability to provide care, that a caregiver may have physical or other limitations, and that it is extremely important to tell the discharge staff about them. One question it suggests asking: does he need help at night and, if so, how will I get enough sleep? [4]

A follow-up list with a name on every line

Start from what the professionals have already told you. Medicare's checklist says to ask for written discharge instructions you can read and understand, to write down your questions and discuss them with the staff, and to write down the names and phone numbers of people to call with questions or concerns. [1]

Anything about falls or medicines goes on the list as a question, not a conclusion. The CDC tells family caregivers to talk openly with the healthcare provider about fall risks. MedlinePlus says to ask the provider all of your questions about the medicines he takes, and to get to know his pharmacist. [7] [8]

- Beside each concern, write who is raising it and with whom.
- A question you've handed to the care team has been referred. It isn't cleared until they answer.

What this means for your family

- “Everyone will pitch in” becomes a page: the parts of the day, a name beside each, and the hours with no name.
- An offer becomes coverage when it has a day and a time on it. Until then it's an offer, and a kind one.
- The open hours and your own limits are information for the care team. They aren't a verdict on the plan, or on you.
- The safety questions aren't yours to answer. Your part is to write them down, hand them to the right person and note who is following up.

What this guide doesn't do

It doesn't say whether the arrangement is safe, how much supervision your husband needs, or whether he can be on his own at any hour. The lists and the advice quoted come from the sources below. The map of the day, and the line between an offer and coverage, are the Family Care Roadmap's own.

Ready for the next step?

You can tell an offer from coverage now, and you know where the open hours and the open questions go. The care interview asks about your husband's needs, his safety and each person who helps, one area at a time, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Centers for Medicare & Medicaid Services, Your Discharge Planning Checklist (CMS Product No. 11376)
2. Administration for Community Living, Who Will Provide Your Care?
3. Centers for Disease Control and Prevention, Check for Safety: A Home Fall Prevention Checklist for Older Adults (STEADI brochure, 2018)
4. Family Caregiver Alliance, Hospital Discharge Planning: A Guide for Families and Caregivers
5. Family Caregiver Alliance, Taking Care of YOU: Self-Care for Family Caregivers
6. AARP, Prepare to Care: A Planning Guide for Families
7. Centers for Disease Control and Prevention, Family Caregivers: Protect Your Loved Ones from Falling (STEADI brochure, 2018)
8. MedlinePlus (National Library of Medicine), Caregiving - medication management: MedlinePlus Medical Encyclopedia